**This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.**

Patient Group Direction
for the supply of Levonorgestrel 1.5mg tablets (Levonelle) and Ulipristal acetate 30mg tablets (ellaOne) for Emergency Contraception

Patient Group Direction, version 006, issued 30 September 2026. Supersedes Version 005, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

## Summary of NICE / NICE CKS Guidelines for Emergency Contraception

### Overview

Emergency contraception is used to prevent pregnancy after unprotected sexual intercourse or contraceptive failure.

### Assessment

The following assessment factors should be considered:

- Time since unprotected sexual intercourse (UPSI)
- Previous UPSI in the cycle
- Current contraception use
- Date of last menstrual period
- Regular cycle length
- Potential drug interactions (enzyme inducers)

- BMI and weight
- Breastfeeding status

**Management**

- **Levonorgestrel:** Within 72 hours (3 days) of UPSI; most effective within 12 hours
- **Ulipristal acetate:** Within 120 hours (5 days) of UPSI
- Consider copper IUD as most effective emergency contraception option (refer to GP/clinic)

**Red flags**

- Suspected existing pregnancy
- Severe asthma insufficiently controlled by oral glucocorticoids (ulipristal)
- Repeated emergency contraception requests (signpost to ongoing contraception)

# Patient Group Direction

for the supply of

**Levonorgestrel 1.5mg tablet (Levonelle)**

for the treatment of

**Emergency Contraception**

**Healthcare professionals covered and training**

| | **Healthcare professionals allowed to work under this PGD and requirements** |
|---|---|
| **Qualifications and Registration** | • Pharmacist registered and practicing with the GPhC<br>• Pharmacy technician registered and practicing with the GPhC |
| **Training and assessment** | Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team.<br>• All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines<br>• All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required<br>• All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to |

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| | be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.<br>• Must work in compliance with the SOPs of their own employer<br>• All users must have appropriate indemnity in place to cover this service |
| **Further training and responsibilities** | • All users must be up to date with CPD requirements and appraisal<br>• All users must be up to date with MHRA and safety alerts with regards to medical products<br>• All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005. |

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| **PGD Indication** | Emergency contraception within 72 hours of unprotected sexual intercourse |
| **Inclusion criteria** | Females of reproductive age, aged 13 and over. Under 13: any sexual activity is a safeguarding concern; do not supply under this PGD, refer to the GP or sexual health service the same day (emergency contraception is time critical) and make a safeguarding referral. Aged 13 to 15: assess and record Fraser competence, ask about coercion, the age of the partner and any safeguarding concern, and follow the local safeguarding pathway. Record the assessment.<br>Within 72 hours of unprotected sexual intercourse (UPSI) or contraceptive failure<br>Valid informed consent obtained<br>No known or suspected pregnancy |
| **Exclusion criteria** | Known or suspected pregnancy<br>Aged under 13. Not supplied under this PGD: same-day referral and a safeguarding referral.<br>Hypersensitivity to levonorgestrel or any component of the formulation<br>Severe hepatic impairment (e.g., cirrhosis)<br>Hereditary galactose intolerance<br>Acute intermittent porphyria |
| **Cautions** | Enzyme-inducing drugs in the last 4 weeks (anticonvulsants, rifampicin, antiretrovirals, St John's Wort): offer a copper IUD; if declined give levonorgestrel 3 mg (two tablets, licensed). Do NOT |

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| | switch to ulipristal, which is not recommended in these women.<br>Malabsorption conditions affecting drug absorption (e.g., Crohn's disease)<br>History of ectopic pregnancy<br>Breastfeeding - avoid for 8 hours after dose<br>Weight 70 kg or over, or BMI 26 or over: ulipristal is preferred (FSRH) unless unsuitable; where levonorgestrel is used give 3 mg (double dose), which is off-label per FSRH guidance. Explain this and record it.<br>Recent unprotected intercourse where pregnancy risk is higher (e.g., mid-cycle) |
| **Actions if patient is excluded or declines treatment** | Advise on alternative treatment options and how these can be accessed.<br>Document any advice given and the decision reached. Inform or refer to the GP as appropriate. |

***Details of the medicine***

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| **Name, form and strength of medicine** | Levonorgestrel 1.5mg tablet (Levonelle) |
| **Legal category** | P (Pharmacy medicine) for women aged 16 and over; POM under 16. This PGD is the legal authority for supply to those under 16; for those 16 and over it sets the standard of assessment and record for what is otherwise a P sale. |
| **Storage** | Store below 25°C in a dry place. Keep in original packaging. |
| **Route/method of administration** | Oral - swallow tablet whole with water |
| **Dose and frequency** | Standard dose: 1.5mg as a single dose, taken as soon as possible after UPSI (ideally within 12 hours, but effective up to 72 hours)<br>Double dose: 3 mg where taking enzyme-inducing drugs (licensed), or where weight is 70 kg or over or BMI 26 or over (off-label, FSRH). Record the reason. |
| **Quantity to be administered and/or supplied** | 1 tablet (1.5mg single treatment) or 2 tablets (3mg if double dose indicated) |
| **Maximum or minimum treatment period** | Single dose only; repeated use not recommended within the same cycle |
| **Adverse effects** | Nausea and vomiting (if vomiting occurs within 3 hours of taking the tablet, another tablet must be taken)<br>Abdominal pain and abdominal discomfort<br>Headache |

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| | Fatigue and tiredness<br>Dizziness and vertigo<br>Breast tenderness<br>Menstrual irregularity (period may be delayed or early)<br>Mood changes (rare)<br>Pelvic pain |

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| **Yellow card reporting** | Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate. |
| **Records to be kept** | Record the following information:<br>• that valid informed consent was given<br>• name of individual, address, date of birth and GP<br>• name of healthcare practitioner<br>• name and brand of medication<br>• date of supply dose, form and route quantity supplied<br>• advice given, including advice given if excluded or declines treatment<br>• details of any adverse drug reactions and actions taken supplied via PGD<br>Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made.<br>A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows:<br>• Adults aged 18 years and over - keep records for 8 years<br>• Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday) |

***Patient information***

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| **Written information to be given to patient or carer** | Supply the patient information leaflet (PIL) provided with the medication. |
| **Follow-up advice to be given to patient or carer** | If vomiting occurs within 3 hours of taking the tablet, return immediately as another tablet is needed.<br>Your next period may be earlier or later than expected. If your period is more than 7 days late, perform a pregnancy test or contact your doctor.<br>Use barrier contraception (condoms) until your next period to reduce risk of pregnancy. |

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| | Consider starting or continuing regular contraception straight away; use condoms until it is reliable. Discuss options with your doctor or healthcare provider.<br><br>This emergency contraception does not protect against sexually transmitted infections (STIs). Discuss STI testing if appropriate.<br><br>If you experience any adverse effects or have concerns, contact your doctor or healthcare provider.<br><br>Remember that emergency contraception works best when taken as soon as possible after unprotected sexual intercourse. |

## Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Emergency Contraception
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

### Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

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| Name and address of pharmacy premises / clinic premises to which this PGD relates | | | |
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| Name of healthcare professional | Job title | Registration number | Signature |
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| Valid from date | Expiry date | | |
| 30/9/26 | 31/7/27 | | |

**PGD adoption and authorisation by the employer/clinical lead**

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

| Name of superintendent / clinical lead | Job title & name of organisation | Signature | Date |
|---|---|---|---|
| | | | |
| Name of professional group signatory | Job title & name of organisation | Signature | Date |
| | | | |

**Change history to version 001**

Later versions are recorded in the Version and change record at the end of this document.

| **Version number** | **Change details** | **Date** |
|---|---|---|
| 001 | Development & issue of new PGD | 1st November 2025 |
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# Patient Group Direction

for the supply of

**Ulipristal acetate 30mg tablet (ellaOne)**

for the treatment of

**Emergency Contraception**

**Healthcare professionals covered and training**

| | **Healthcare professionals allowed to work under this PGD and requirements** |
|---|---|
| **Qualifications and Registration** | • Pharmacist registered and practicing with the GPhC<br>• Pharmacy technician registered and practicing with the GPhC |
| **Training and assessment** | Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team.<br>• All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines<br>• All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required<br>• All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.<br>• Must work in compliance with the SOPs of their own employer<br>• All users must have appropriate indemnity in place to cover this service |
| **Further training and responsibilities** | • All users must be up to date with CPD requirements and appraisal<br>• All users must be up to date with MHRA and safety alerts with regards to medical products |

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| | • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005. |

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| **PGD Indication** | Emergency contraception within 120 hours of unprotected sexual intercourse |
| **Inclusion criteria** | Females of reproductive age, aged 13 and over. Under 13: any sexual activity is a safeguarding concern; do not supply under this PGD, refer to the GP or sexual health service the same day (emergency contraception is time critical) and make a safeguarding referral. Aged 13 to 15: assess and record Fraser competence, ask about coercion, the age of the partner and any safeguarding concern, and follow the local safeguarding pathway. Record the assessment.<br><br>Within 120 hours (5 days) of unprotected sexual intercourse (UPSI) or contraceptive failure<br><br>Valid informed consent obtained<br><br>No known or suspected pregnancy |
| **Exclusion criteria** | Known or suspected pregnancy<br><br>Aged under 13. Not supplied under this PGD: same-day referral and a safeguarding referral.<br><br>Hypersensitivity to ulipristal acetate or any component of the formulation<br><br>Severe hepatic impairment (e.g., cirrhosis)<br><br>Severe asthma insufficiently controlled by oral glucocorticoids<br><br>Hereditary galactose intolerance<br><br>Enzyme-inducing drugs in the last 4 weeks (anticonvulsants, rifampicin, antiretrovirals, St John's Wort): ulipristal is not recommended (SmPC). Offer a copper IUD or levonorgestrel 3 mg. |
| **Cautions** | Progestogen-containing contraceptive taken in the previous 7 days: may reduce ulipristal efficacy; consider levonorgestrel instead. Hormonal contraception must not be restarted until 5 days after ulipristal, with condoms until it is reliable again.<br><br>Malabsorption conditions affecting drug absorption<br><br>History of ectopic pregnancy<br><br>Breastfeeding - avoid breastfeeding for 7 days after dose<br><br>Repeated use in the same cycle not recommended<br><br>Weight 70 kg or over, or BMI 26 or over: ulipristal is the preferred oral option (FSRH). No dose adjustment is needed. |
| **Actions if patient is excluded or declines treatment** | Advise on alternative treatment options and how these can be accessed. |

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| | Document any advice given and the decision reached. Inform or refer to the GP as appropriate. |

***Details of the medicine***

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| **Name, form and strength of medicine** | Ulipristal acetate 30mg tablet (ellaOne) |
| **Legal category** | P (Pharmacy medicine). ellaOne is licensed for sale without prescription at any age of reproductive potential; this PGD sets the standard of assessment and record for the supply. |
| **Storage** | Store below 25°C. Keep in original packaging. |
| **Route/method of administration** | Oral - swallow tablet whole with water |
| **Dose and frequency** | 30mg as a single dose, taken as soon as possible after UPSI (effective up to 120 hours/5 days) |
| **Quantity to be administered and/or supplied** | 1 tablet (30mg single treatment) |
| **Maximum or minimum treatment period** | Single dose only; repeated use not recommended within the same cycle |
| **Adverse effects** | Headache<br>Nausea<br>Abdominal pain and abdominal discomfort<br>Dysmenorrhoea (painful period)<br>Fatigue and tiredness<br>Dizziness and vertigo<br>Mood changes and mood swings<br>Myalgia (muscle pain)<br>Back pain<br>Breast tenderness<br>Menstrual irregularity<br>Pelvic pain |

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| **Yellow card reporting** | Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate. |
| **Records to be kept** | Record the following information:<br>• that valid informed consent was given |

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|---|---|
| | • name of individual, address, date of birth and GP<br>• name of healthcare practitioner<br>• name and brand of medication<br>• date of supply dose, form and route quantity supplied<br>• advice given, including advice given if excluded or declines treatment<br>• details of any adverse drug reactions and actions taken supplied via PGD<br>Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made.<br>A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows:<br>• Adults aged 18 years and over - keep records for 8 years<br>• Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday) |

***Patient information***

| | |
|---|---|
| **Written information to be given to patient or carer** | Supply the patient information leaflet (PIL) provided with the medication. |
| **Follow-up advice to be given to patient or carer** | If vomiting occurs within 3 hours of taking the tablet, return immediately as another tablet is needed.<br>Your next period may be earlier or later than expected. If your period is more than 7 days late, perform a pregnancy test or contact your doctor.<br>Use barrier contraception (condoms) until your next period to reduce risk of pregnancy.<br>Do not start or restart hormonal contraception until 5 days after taking ulipristal, and use condoms until it is reliable. Discuss options with your doctor or healthcare provider.<br>This emergency contraception does not protect against sexually transmitted infections (STIs). Discuss STI testing if appropriate.<br>If you experience any adverse effects or have concerns, contact your doctor or healthcare provider.<br>Remember that emergency contraception works best when taken as soon as possible after unprotected sexual intercourse. |

## Key references


- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2

- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Emergency Contraception
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

**Agreement to practise by the Registered Healthcare Professional**

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

| | | | |
|---|---|---|---|
| Name and address of pharmacy premises / clinic premises to which this PGD relates | | | |
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| Name of healthcare professional | Job title | Registration number | Signature |
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| Valid from date | Expiry date | | |
| 30/9/26 | 31/7/27 | | |

**PGD adoption and authorisation by the employer/clinical lead**

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

| Name of superintendent / clinical lead | Job title & name of organisation | Signature | Date |
|---|---|---|---|
| | | | |
| Name of professional group signatory | Job title & name of organisation | Signature | Date |
| | | | |

**Change history to version 001**

Later versions are recorded in the Version and change record at the end of this document.

| **Version number** | **Change details** | **Date** |
|---|---|---|
| 001 | Development & issue of new PGD | 1st November 2025 |
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**Version and change record**

Version: v006

Issued: 30 September 2026

Supersedes: Version 005, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

**Previous version records**

Version: v005

Issued: 14 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Under 13 is not supplied under this PGD in either arm: same-day referral to the GP or sexual health service and a safeguarding referral. The previous text said supply may still be appropriate.

Guidance summary red flag for ulipristal now matches the exclusion: severe asthma insufficiently controlled by oral glucocorticoids.

Ulipristal arm now carries the weight and BMI direction that only the levonorgestrel arm stated.

Duplicate 5 day line removed from the ulipristal cautions.

Follow-up advice on restarting contraception written per arm.

Cover and arm headings corrected from administration to supply.

Broken NICE MPG2 link corrected.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Legal categories corrected: levonorgestrel 1.5 mg is P at 16 and over and POM under 16; ulipristal acetate 30 mg (ellaOne) is P. Version 002 called both POM.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. The ulipristal arm previously carried two expiry dates (31/7/27 and 31/10/26).

Safeguarding: the inclusion criterion admitted girls from age 11 with no safeguarding provision. Under 13 now carries a mandatory safeguarding referral; 13 to 15 requires a recorded Fraser competence assessment and safeguarding questions.

Enzyme inducers: the levonorgestrel caution told the pharmacist to consider switching to ulipristal, which the ellaOne SmPC says is not recommended after enzyme inducer use in the previous 4 weeks. Both arms now direct to a copper IUD or levonorgestrel 3 mg (licensed). Enzyme inducer use is an exclusion for ulipristal.

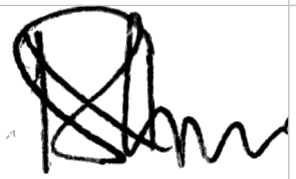
Ulipristal: 'concurrent or recent use of hormonal contraceptives' was an exclusion while 'contraceptive failure' was an inclusion, so every woman presenting after a pill failure was both included and excluded. Replaced with the FSRH position as a caution.

Vomiting: both SmPCs require another tablet if vomiting occurs within 3 hours; the document said 2 hours in three places. Corrected.

Weight-based 3 mg levonorgestrel is now labelled as off-label per FSRH, with ulipristal preferred at 70 kg or over or BMI 26 or over.

## Signed on behalf of Get Real Health

Version v006, 30 September 2026.

| Name | Qualification | Signature | Date |
|---|---|---|---|
| Nitin Shori | Medical Director<br>GMC: 6047293 |  | 30 September 2026 |
| Chris Pilkington | Head Pharmacist<br>GPhC: 2046322 |  | 30 September 2026 |

*Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.*